



Fiona Stanley Fremantle Hospitals Group

Guideline

Hampton House Referral

Reference #: FSFH-MENH-GUI-0009

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

Hampton House is a community based residential facility that provides short term clinical support for persons who are experiencing significant deterioration in their level of functioning due to their mental health. This guideline clearly defines the Hampton House referral process including who can refer, how to refer and the documentation required.

2. Terminology

Assessment & Treatment Team (ATT)	Multidisciplinary community team operating 08:30-16:30 five days a week providing care coordination and short-term skills based biopsychosocial interventions for persons residing in the FSFHG catchment area for a time limited period (up to 10 weeks).
Care Coordinator	The nominated person who has the responsibility for coordinating, facilitating and integrating mental health treatment, care and support to a mental health consumer, their family / carers and significant others. Care is tailored to meet the specific needs of individual consumers.
Mental Health Urgent Care (MHUC)	Multidisciplinary triage team based at Fremantle Hospital Mental Health Service operating from 08:00 to 22:00hrs seven days a week that provides the point of access for all referrals to the adult mental health service.
PSOLIS	Psychiatric Services Online Information System
Severe mental illness	Mental disorders causing significant acute or chronic impairment including but not limited to, schizophrenia, psychosis, bipolar mood disorder, schizoaffective disorder and postpartum psychosis.

3. Guideline

- Hampton House offers:
 - less restrictive, community-based alternative to an acute psychiatric inpatient admission
 - accommodation for up to fifteen adults
 - 24 hours a day clinical staff to provide consumers with support and advice.

3.1. Inclusion criteria

- To be eligible for an admission to Hampton House the consumer must:
 - be active with a mental health service as governance of the patient's treatment remains with their treating team during admission to Hampton House
 - have mental illness as their primary diagnosis.
 - have a diagnosed severe and enduring mental illness and be experiencing an episode that significantly impairs their ability to live in the community.
 - have reduced capability for independent living to such an extent that hospitalisation would be probable.
 - aged between 18 and 65 years (exceptions will be individually assessed)
 - have an allocated community care coordinator actively involved in the consumer's ongoing care.

3.2. Exclusion criteria

- Consumers will be excluded from admission to Hampton House where:
 - mental illness is not the consumers primary concern e.g. alcohol and other drug issue are primary concern.
 - admission is required to alleviate accommodation and/or financial difficulties.
 - the consumer's behaviour is likely to negatively impact the wellbeing of other consumers of the service.
 - a risk assessment indicates the consumer cannot be safely accommodated at Hampton House.
 - a high level of assistance is required for activities of daily (including personal care
 - there are complex equipment requirements due to the consumers compromised mobility.

3.3. Referrals

- The multidisciplinary team (MDT) of referring team (both Inpatient and Outpatient) will initially discuss referral for admission to Hampton House
- The referring clinician will then ensure:
 - the consumer meets the inclusion criteria.
 - the consumer and carer have been informed, involved as a partner in this decision and provide verbal consent for the referral.
 - an E-referral has been completed, providing rationale if requesting a single room.
 - the consumer and carer have been provided with the Hampton House Consumer information pamphlet.
 - the PSOLIS Treatment Support and Discharge Plan has been updated to reflect referral to Hampton House
- The consumer will not be transferred or directed to the Hampton House until Hampton House clinicians have confirmed the referral has been accepted and the admission time has been agreed.
- The referring clinician will ensure the following documentation is provided to Hampton House prior to the admission:
 - demographics
 - current diagnosis
 - recent history
 - mental state examination
 - hospital medication chart
 - medication profile
 - agitation and arousal chart
- Consumers referred to Hampton House for step down admission who require short term follow up by the Assessment and Treatment Team (ATT) will be discussed on an individual basis by ATT and the Hampton House Clinical Nurse Consultant.

3.4. Hampton House clinicians

- On receipt of the E-referral:
 - Monday – Friday business hours the Clinical Nurse Consultant reviews and accept/decline.
 - After hours (to 20:00hrs) nursing staff on duty review and accept/decline
- Hampton House clinicians will:

- provide relevant feedback to the referrer if the referral does not meet the inclusion criteria and is not accepted.
- liaise with the referring clinician to progress the referral if accepted.
- determine planned length of stay as part of overall bed management process.
- ensure required documentation is complete and received including Risk Assessment and Management Plan and Treatment Support and Discharge Plan
- triage the referral and email patient demographics to the FH Mental Health Urgent Care Triage reception to enter details in webPAS and generate patient identification labels.
- complete the Hampton House referral and activation online form.
- confirm with the referrer the date and time of admission.
- Consumers will not be admitted to Hampton House after 20:00hrs.

4. Compliance/Performance Monitoring

- Any clinical incident related to referral or admission to Hampton House will be reported through the Datix Clinical Incident Management System and monitored by the Hampton House Clinical Nurse Consultant.
- Complaints related to referral or admission to Hampton House will be actively monitored and addressed by the Program Manager and reported to the FSFH Clinical Governance Sub Committee.

5. Related Policy Documents

- WA Health:
 - [WA Health Consent to treatment policy 2016](#)
 - [Statewide Standardised Clinical Documentation for Mental Health Services](#)
 - [Clinical Incident Management Policy](#) MP 0122/19
- SMHS
 - [Care coordination in mental health](#)
- FSFHG
 - Infection Prevention and Management: COVID-19 (FSFH-HW_POL_0085)
 - Clinical risk assessment and management (FSFH-HW-POL-0017)
 - Clinical Handover

6. Related Standards

- NSQHS standards:
 - Clinical Governance
 - Partnering with Consumers
 - Communicating for safety
- Chief Psychiatrists Standards for clinical care:
 - Care planning.

7. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	07/2016	CNC, Hampton Road Service	FHMHS Clinical Governance Committee	FHMHS Clinical Governance Committee	07/2019
2	10/2019	CNC, Hampton House	NMPGC	FSFHG Policy Committee	10/2021
3	01/2022	CNC, Hampton House	S5 SQRC, consumer consultation	Service 5, Safety and Quality Committee	01/2025
4	03/2025	CNC Hampton House	NMPGC, MH Clinical Governance Sub Committee	Service 5, Safety and Quality Committee	04/2028